8. 兒童心臟內科常見的檢查及其參考數值

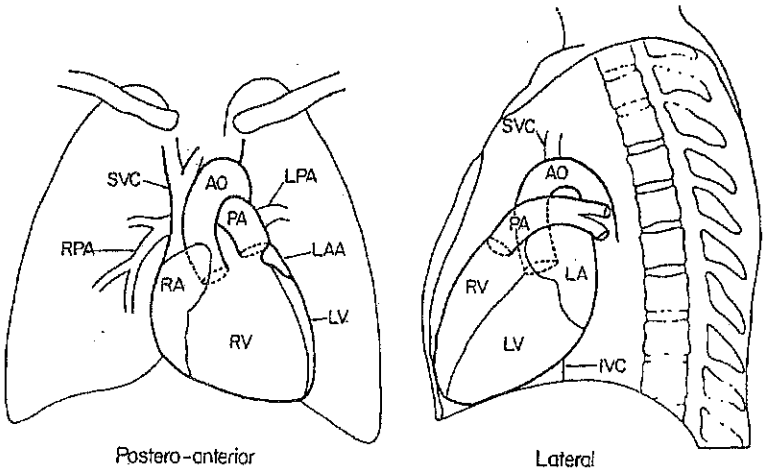


FIG. 4–2.
PA and lateral projections of normal cardiac silhouette. Note that in the lateral projection, the RV is contiguous with the lower third of the sternum and that the LV normally crosses the posterior margin of the IVC above the diaphragm. *AO,* aorta; *LPA,* left pulmonary artery; *RPA,* right pulmonary artery; *IVC,* inferior vena cava; *LA,* left atrium; *LAA,* left atrial appendage; *LV,* left ventricle; *PA,* pulmonary artery; *RA,* right atrium; *RV,* right ventricle; *SVC,* superior vena cava.

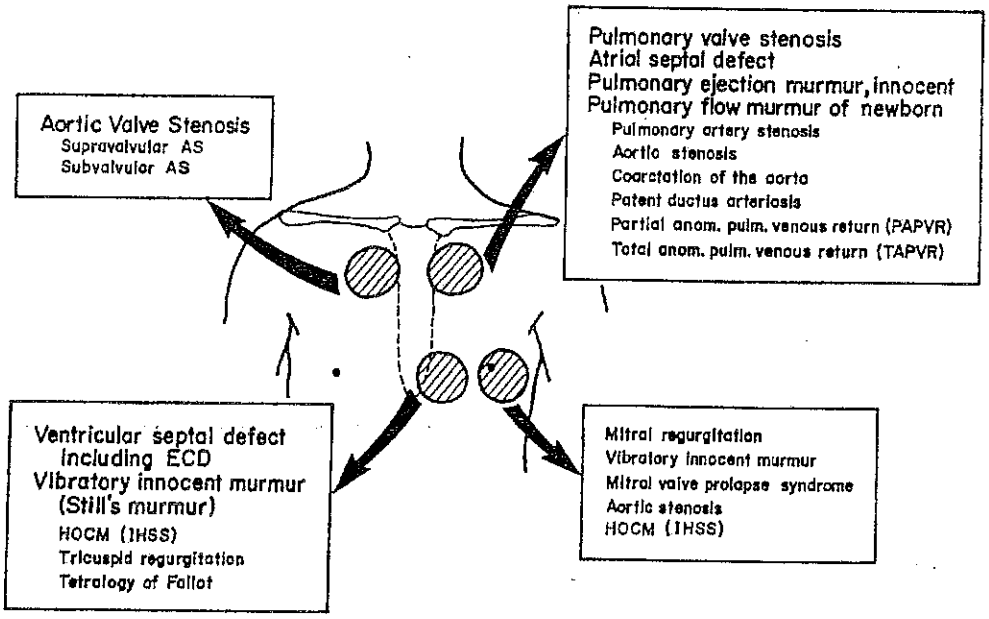


FIG. 2–8.
Diagram showing systolic murmurs audible at various locations. Less common conditions are shown in smaller type (see Tables 2-6 through 2-9). *IHSS,* Idiopathic hypertrophic subaortic stenosis; *AS,* aortic stenosis, *ECD,* endocardial cushion defect; *HOCM,* hypertrophic obstructive cardiomyopathy.

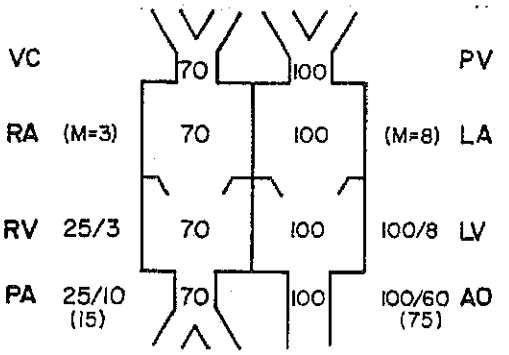


FIG. 7-1.
Pressure and oxygen saturation values in normal children.

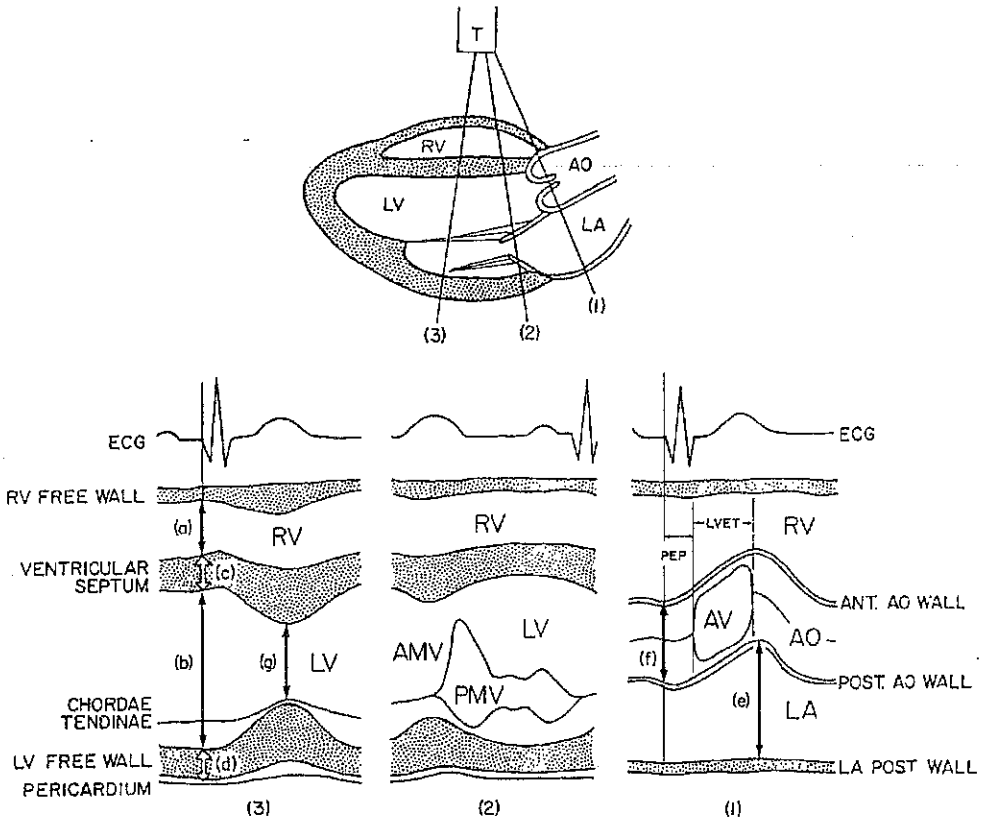


FIG. 6-1.
A cross-sectional view of the left side of the heart along the long axis *(top)* through which "ice-pick" views of the M-mode echo recordings are made *(bottom)*. Many other M-mode views are possible, but only three are shown in this figure. The dimension of the aorta *(AO)* and left atrium is measured along the line *(1)*. Systolic time intervals for the left side are also measured at the level of the aortic valve *(AV)*. The line *(2)* passes through the mitral valve. Measurements made at this level are not useful in pediatric patients. Measurement of chamber dimensions and wall thickness of right and left ventricles is made along the line *(3)*. Normal values of these measurements are shown in Table 6-1. *(a)*, RV dimension; *(b)*, LV diastolic dimension; *(c)*, interventricular septal thickness; *(d)*, LV posterior wall thickness; *(e)*, LA dimension; *(f)*, aortic dimension; *(g)*, LV systolic dimension; *AMV*, anterior mitral valve; *LVET*, left ventricular ejection time; *PEP*, preejection period; *PMV*, posterior mitral valve.

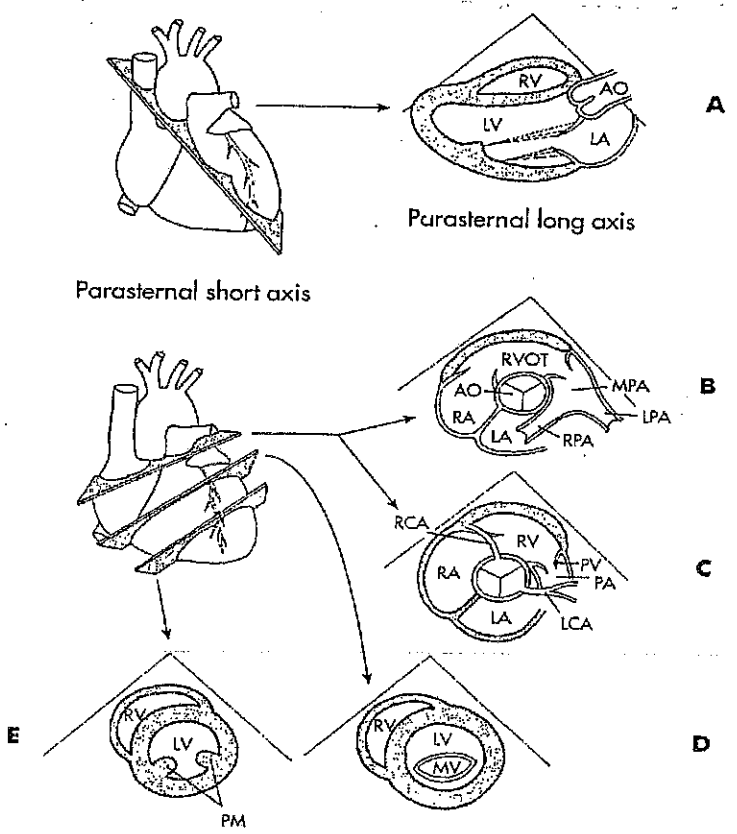


FIG. 6-2.
Diagrammatic illustration of important two-dimensional echo views obtained from the parasternal transducer position. Parasternal long-axis view **(A)** is shown at the top. Parasternal short-axis views obtained at various levels: the semilunar valve and great artery level **(B, C)**, the mitral valve level **(D)**, and the papillary muscle level **(E)**. *AO,* Aorta; *MV,* mitral valve; *LCA,* left coronary artery; *LPA,* left pulmonary artery; *PM,* papillary muscle; *RCA,* right coronary artery.

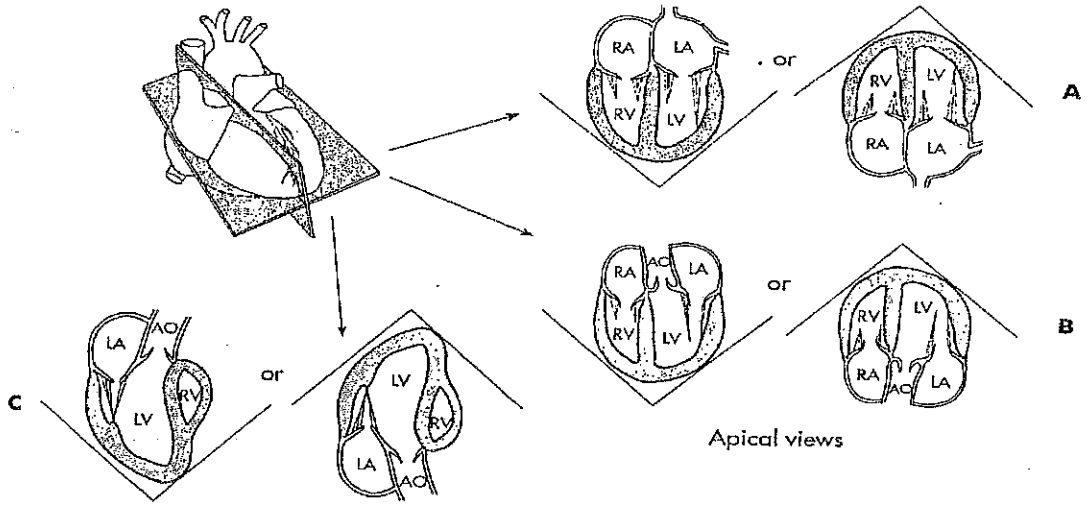


FIG. 6-3.
Diagrammatic illustration of two-dimensional echo views obtained with the transducer at the apical position. Both the apex-down and apex-up images are shown. **A,** Apical four-chamber view. **B,** Apical four-chamber view with LV outflow tract (apical "five-chamber" view). **C,** Apical long-axis view.

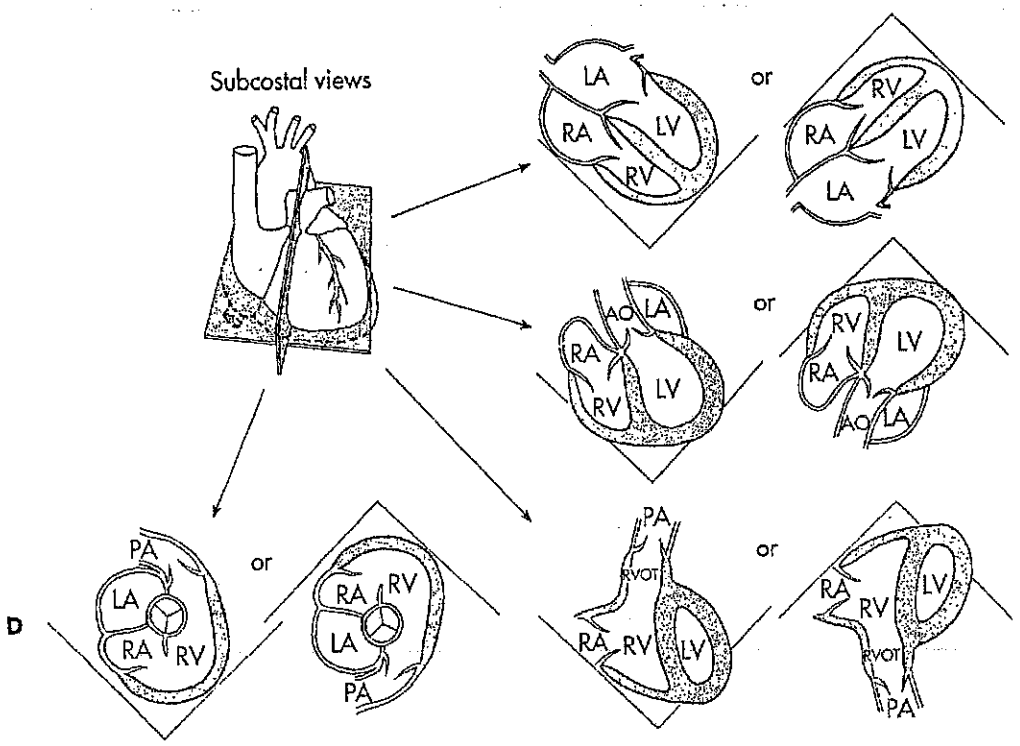


FIG. 6-4.
Diagrammatic illustration of two-dimensional echo views obtained with the transducer at the subcostal position. Both the apex-down and apex-up images are shown. **A,** subcostal four-chamber view. **B,** View showing the LV outflow tract and the proximal aorta (subcostal "five-chamber" view). **C,** View that shows the RV outflow tract and the proximal MPA. **D,** Subcostal short-axis view. LPV, Left pulmonary vein; *RPV,* right pulmonary vein.

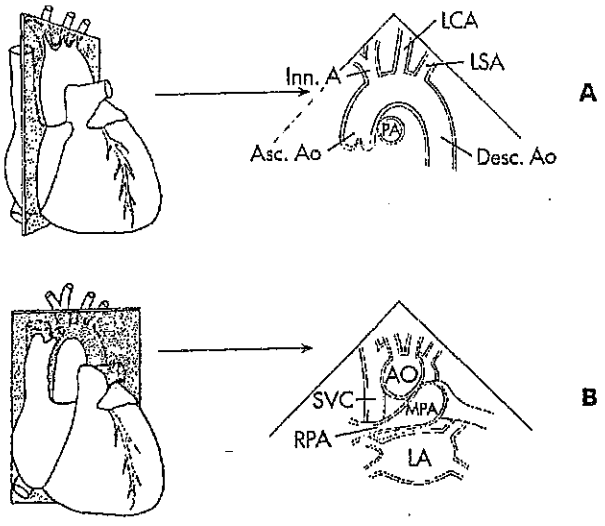


FIG. 6-5.
Diagrammatic drawing of SSN two-dimensional echo views. **A,** Long-axis view. **B,** Short-axis view. *AO,* Aorta; *Asc Ao,* ascending aorta; *Desc Ao,* descending aorta; *Inn A,* innominate artery; *LCA,* left carotid artery; *LSA,* left subclavian artery.